Assignment 6

Name: Rhys Brown

Triage Pseudocode

1. After the patient enters the ED, the attending nurse/physician comes to them with a hospital owned tablet containing the triage model.
2. The medical professional immediately observes the patient. If immediate life-threatening conditions such as a heart attack or a lack of breathing are observed, the patient is immediately rushed for treatment without going through the rest of the digital triage process.
3. If no immediate life-threatening conditions are observed the patient is assessed for: Age, Gender, GCS, SBP, DBP, Pulse, Temperature, Respiratory Rate, Sp02 and purpose for visit. Shock index and MAP are calculated from these values; then, an ID number is assigned to the patient.
4. These attributes are fed to a pre-trained ML/DL algorithm running locally on the device. The information is then stored on the hospital's records in a secure database.

Table showing some metrics/criterion used to classify triage levels

Metrics/Criterion	**Triage level**
Patient in immediate risk of death (skip rest of tests and treat immediately).	Level 1
Patient critical but can survive without immediate treatment. They likely have: **GCS** 9–12 (diminished mental status but can protect airway) **SBP:** 90–100 mmHg **DBP:** 60–70 mmHg **Pulse** > 100 bpm **Temperature**: 38.5–40°C (dangerously high fever)	Level 2

Respiratory Rate > 20 **SpO_2:** 90–94% (moderate hypoxia) **MAP**: 65–75 mmHg (mild perfusion difficulties)	
Patient suffering from immediate acute illness/injury but otherwise stable.	Level 3
Patient needs a minor procedure. ie stitches.	Level 4
Patient is in for routine reason	Level 5

5. A triage score is assigned to the patient by the ML/DL algorithm. The patient then receives an identification band.
6. The patient is escorted to the waiting area and is seen according to their triage level.